

Clinical Study Summary Report (CSSR)

Document Status: Final | Version: 1.0 | Date: 01-APR-2026

1. Administrative & Study Identification

Full Study Title:	A PROSPECTIVE, RANDOMIZED, OPEN-LABEL PHASE 2 STUDY TO EVALUATE THE SUPERIORITY OF INOTUZUMAB OZOGAMICIN MONOTHERAPY VERSUS ALLR3 FOR INDUCTION TREATMENT OF CHILDHOOD HIGH RISK FIRST RELAPSE B-CELL PRECURSOR ACUTE LYMPHOBLASTIC LEUKAEMIA
Short Title:	A Study to Learn More About the Study Medicine Called Inotuzumab Ozogamicin (InO) in Children (1 to <18 Years) With First Relapse ALL
Protocol Number:	987429954398390850
NCT Number:	NCT05748171
Sponsor Name:	Pfizer
Investigational Product:	inotuzumab ozogamicin
Phase:	PHASE2
Medical Monitor:	[Not Provided in Posting]

2. Study Synopsis & Rationale

2.1 Study Objective

Primary Objective:	1. Minimum Residual Disease (MRD) Negativity in participants achieving complete response (CR), complete response with incomplete platelet count recovery (CRp), or complete response with incomplete count recovery (CRi): MRD negativity status is determined based on the minimum MRD percentage between the date of CR/CRp/CRi and end of treatment test as assessed by RQ-PCR, with reflex to FC result if MRD is non-evaluable by RQ-PCR
Secondary Obj.:	1. Event Free Survival (EFS) 2. Duration of Response (DoR) for Participants Who Achieved CR/CRp/CRi 3. Rate of hematopoietic stem cell transplantation (HSCT)

2.2 Background & Rationale

This prospective, randomized, multicenter, open-label Phase 2 study is designed to evaluate the superiority of InO monotherapy vs ALLR3 after 1 cycle of induction treatment in paediatric participants (between 1 and <18 years) with High Risk (HR) first bone marrow relapse CD22-positive BCP ALL, and to evaluate the safety and tolerability, PK and long-term efficacy. Treatment with study intervention will end after induction therapy; follow-up will continue for up to 5 years from randomization.

3. Study Design & Methodology

Design Framework:	Type: INTERVENTIONAL, Phase: PHASE2, Status: RECRUITING
Target N:	100

4. Subject Selection (Eligibility Criteria)

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4.1 Inclusion Criteria

1. Male or female participants between 1 and <18 years of age.
2. Morphologically confirmed diagnosis of first relapse HR BCP ALL; HR first relapse is defined as relapse occurring within 18 to 30 months of original diagnosis of ALL or within 6 months of completion of primary therapy, and lacking any identified very high-risk genetic abnormalities (Groeneveld-Krentz et al, 2019) (ie, KMT2A::AFF1 fusion $t(4;11)(q21;q23)$, TCF3-HLF fusion $t(17;19)(q22;p13)$, TCF3-PBX1 fusion $t(1;19)(q23;p13.3)$, hypodiploidy <40 chromosomes) or masked low hypodiploidy (Molina et al, 2021), TP53 alteration).
 - * CD22-positive ALL as defined by local institution;
 - * Bone marrow involvement of $\geq 5\%$ leukemic blasts ($\geq M2$ status).
3. Adequate serum chemistry parameters:
 - * An eGFR in participants 1 to <2 years of age, or eCrCl in those 2 to <18 years of age, ≥ 30 mL/min using the recommended formula in Section 10.10.2.
 - * AST and ALT $\geq 5 \times$ institutional ULN at the time of randomization or pre-cytoreduction/general anesthesia;
 - * Total bilirubin $\geq 1.5 \times$ institutional ULN unless the participant has documented Gilbert's syndrome;
4. Prior history of thrombosis during corticosteroid use and/or asparaginase are eligible provided the patient receives anti-coagulant prophylaxis per institutional guidelines.
5. Cardiac shortening fraction $\geq 30\%$ by echocardiogram or ejection fraction $\geq 50\%$ by MUGA.
6. Participants with combined bone marrow and testicular relapse are eligible assuming orchiectomy is performed prior to randomization or is planned at the end of induction therapy.

5.2. Exclusion Criteria

1. Any history of prior or ongoing hepatic SOS or prior liver failure [defined as severe acute liver injury with encephalopathy and impaired synthetic function (INR of ≥ 1.5)].
2. Prior allo-HSCT or CAR T-cell therapy.
3. Isolated extramedullary leukemia.
4. Philadelphia-chromosome positive ALL, ie. BCR-ABL/t(9;22) present.
5. Prior therapy with a calicheamicin-conjugated antibody (eg, InO or gemtuzumab ozogamicin).
6. Participants with active, uncontrolled bacterial, fungal, or viral infection.
7. Hypersensitivity/allergy to both PEG-ASP and Erwinia-ASP

4.2 Exclusion Criteria

Not Provided

11. Study Identifiers & Governance

Primary ID:	987429954398390850
Registry Number:	NCT05748171
Sponsor:	Pfizer

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15. Sign-off & Approval

Principal Investigator: _____ Date: _____

Clinical Operations Lead: _____ Date: _____

Lead Statistician: _____ Date: _____